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Surgical drape support

Abstract

A surgical drape support for positioning on a patient's chest to support a drape above the patient's nose and mouth includes a main arch defining an inner main surface, an outer main surface, a main crown, a first main base edge, and a second main base edge. A first lower foot extends downward from the first main base edge and away from the outer main surface. A second lower foot extends downward from the second main base edge and away from the outer main surface. A first upper foot and a second upper foot extend upwards and away from the outer main surface to a position above the main crown. When the first lower foot and the second lower foot are positioned on the patient's chest they are configured to support the drape over the patient's nose and mouth.

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Background/Summary

CROSS-REFERENCE TO RELATED APPLICATION (1) This application claims priority to and the benefit of the filing date of U.S. Provisional Patent Application No. 63/400,186, filed Aug. 23, 2022, which is incorporated herein by reference in its entirety.

FIELD

(1) This disclosure relates to a surgical drape support for maintaining a surgical drape properly positioned over a patient's nose and mouth. More specifically, the disclosure relates to a surgical drape support that is positionable on the patient's chest and configured to maintain proper orientation of the surgical drape during surgery.

BACKGROUND

(2) The present disclosure describes a support for a surgical drape, for example, an ophthalmic drape and oxygen delivery system. An ophthalmic drape is required during eye surgery to provide a

barrier between the patient and the operating surgeon. Ophthalmic drapes are typically placed over the patient and extend upwards on their body to their nose. Typically, only the eyes of the patient are exposed during the operation. A wide variety of methods have been used to support the ophthalmic drape including drape clips, IV poles, stretcher attachments, kidney basins, and even an auto repair funnel. However, many of these are complex, costly, difficult to install and adjust. In addition, many are not medical grade. The biggest challenge to present designs is proper adjustment such that the ophthalmic drape does not interfere with the patient's nose or mouth. Improper installation or insufficient support can result in limited spacing between the nose and mouth or excess spacing which can interfere with the surgeon's access to the eyes.

(3) For some patients, the close proximity of a surgical drape to their nose and mouth may generate a feeling of claustrophobia and induce panic. Similarly, the attachment of an oxygen mask or cannula can exacerbate the feeling of claustrophobia. This is especially true for patients suffering from post-traumatic stress disorder. For these patients, a support that provides sufficient space and allows oxygen to be delivered to the cavity between the patient and the surgical drape, as opposed to attaching oxygen directly to the patient, would be highly desirable. It would also be highly desirable to have an inexpensive and easily adjustable support that is compatible with different body sizes.

Description

BRIEF DESCRIPTION OF THE DRAWINGS

- (1) FIG. 1 is an isometric view of an example surgical drape support according to the disclosure;
- (2) FIG. 2 is an isometric illustration of the surgical drape support shown in FIG. 1;
- (3) FIG. 3 is a side view of the surgical drape support shown in FIG. 1;
- (4) FIG. 4 is a front view of the surgical drape support shown in FIG. 1;
- (5) FIG. 5 is a bottom view of the surgical drape support shown in FIG. 1;
- (6) FIG. 6 is a side view of the surgical drape support shown in FIG. 1, the surgical drape support shown positioned on the patient's chest;
- (7) FIG. 7 is a side view of the surgical drape support shown in FIG. 1, the surgical drape support shown positioned on the patient's chest and an ophthalmic drape overlaid on the patient;
- (8) FIG. 8 is a side view of the surgical drape support shown in FIG. 1, the surgical drape support shown positioned on the patient's chest and an ophthalmic drape overlaid on the patient and an oxygen supply attached; and
- (9) FIG. 9 is a side view of the surgical drape support shown in FIG. 1, the surgical drape support shown positioned on a smaller patient's chest and an ophthalmic drape overlaid on the patient and an oxygen supply attached.

DETAILED DESCRIPTION

(10) The present disclosure can be understood more readily by reference to the following detailed description, examples, drawings, and claims, and their previous and following description.

However, before the present devices, systems, and/or methods are disclosed and described, it is to be understood that this invention is not limited to the specific devices, systems, and/or methods disclosed unless otherwise specified, as such can, of course, vary. It is also to be understood that the terminology used herein is for the purpose of describing particular aspects only and is not intended to be limiting.

(11) The following description of the invention is provided as an enabling teaching of the invention in its best, currently known embodiment. To this end, those skilled in the relevant art will recognize and appreciate that many changes can be made to the various aspects of the invention described herein, while still obtaining the beneficial results of the present invention. It will also be apparent that some of the desired benefits of the present invention can be obtained by selecting some of the

features of the present invention without utilizing other features. Accordingly, those who work in the art will recognize that many modifications and adaptations to the present invention are possible and can even be desirable in certain circumstances and are a part of the present invention. Thus, the following description is provided as illustrative of the principles of the present invention and not in limitation thereof.

(12) FIGS. **1-5** show surgical drape support **10** in accordance with one aspect of the present disclosure. The surgical drape support **10** includes a main arch **12** defining an inner main surface **14**, an outer main surface **16**, a main crown **18**, a first main base edge **20**, and a second main base edge **22**. In one aspect of the disclosure, the main arch **12** comprises a semi-cylinder shape. The surgical drape support **10** further includes a first lower foot **24** extending downward from the first main base edge **20** and away from the outer main surface **16**. In one aspect of the disclosure the first lower foot **24** is angled down relative to a plane generated by the first main base edge **20** and the second main base edge **22** by 10 to 30 degrees. The angle of the first lower foot **24** is configured to engage the chest **26** of a patient **28** (see FIG. **6**). In one aspect of the disclosure, this comprises having the first lower foot **24** be at least slightly concave so conform to the patient's chest **26**. Similarly, the surgical drape support **10** includes a second lower foot **30** extending downward from the second main base edge **22** and away from the outer main surface **16**. In one aspect of the disclosure, the second lower foot **30** is a mirror of the first lower foot **24**. In one aspect of the disclosure the second lower foot **30** is angled down relative to a plane generated by the first main base edge **20** and the second main base edge **22** by 10 to 30 degrees. The angle of the second lower foot **30** is configured to engage the chest **26** of a patient **28** (see FIG. **6**). In one aspect of the disclosure, this comprises having second first lower foot **30** be at least slightly concave so conform to the patient's chest **26**.

(13) The surgical drape support **10** may further include a first upper foot **32** extending upward and away from the outer main surface **16** to a position above the main crown **18**. In one aspect of the disclosure the first upper foot **32** is angled upward relative to a plane generated by the first main base edge **20** and the second main base edge **22** by 10 to 30 degrees. The angle of the first upper foot **32** is configured to engage the chest **26** of a patient **28** (see FIG. **9**). In one aspect of the disclosure this comprises having the first upper foot **32** be at least slightly concave when the first upper foot **32** is in contact with the patient's chest **26** (see FIG. **9**). In another aspect of the disclosure, this comprises having the first upper foot **32** be at least slightly convex when the first lower foot **24** and the second lower foot **30** are in contact with the patient's chest **26** (FIG. **6**). Similarly, the surgical drape support **10** may further include a second upper foot **34** extending upward and away from the outer main surface **16** to a position above the main crown **18**. In one aspect of the disclosure, the second upper foot **34** is a mirror of the first upper foot **32**. In another aspect, the second upper foot **34** is angled upward relative to the plane generated by the first main base edge **20** and the second main base edge **22** by 10 to 30 degrees. The angle of the second upper foot **34** is configured to engage the chest **26** of the patient **28** (see FIG. **9**). In one aspect of the disclosure this comprises having the second upper foot **34** be at least slightly concave when the second upper foot **34** is in contact with the patient's chest **26** (see FIG. **9**). In another aspect of the disclosure, this comprises having the second upper foot **34** be at least slightly convex when the first lower foot **24** and the second lower foot **30** are in contact with the patient's chest **26**.

(14) In one aspect of the disclosure, the first lower foot **24** and the second lower foot **30** define a lower foot width **36** (see FIG. **4**). Similarly, the first upper foot **32** and the second upper foot **34** define an upper foot width **38**. In one aspect, the upper foot width **38** is smaller than the lower foot width **36**. This allows the surgical drape support **10** to be inverted between a position wherein the first lower foot **24** and second lower foot **30** are in contact with the patient's chest (FIGS. **6-8**) when the patient **28** is larger to a position wherein the first upper foot **32** and the second lower foot **34** are in contact with the patient's chest **26** (FIG. **9**) when the patient **28** is smaller. In one aspect, the lower foot width **36** is approximately 202 mm and the upper foot width **38** is approximately 138

mm. In another aspect, the lower foot width **36** is between 180 mm and 220 mm and the upper foot width **38** is between 118 mm and 158 mm. In still another aspect, the upper foot width **38** is approximately 70% of the lower foot width **36**. Although specific dimensions and ranges are described, it is contemplated that the specific sizing of the lower foot width **36** and the upper foot width **38** may be tailored to the average sizes of larger and smaller patients **28**. The main arch **12** defines a main arch height **40** (FIG. 3). In one aspect of the disclosure the main arch height **40** is approximately 85 mm. In another aspect, the main arch height **40** is between 40 percent and 50 percent of the lower foot width **36**.

(15) Although one aspect of the disclosure completes the use of a symmetrical surgical drape support **10**, in another aspect shown in FIGS. 2 and 3 the surgical drape support **10** is comprised of a relatively symmetrical body portion **42** and a front lip portion **44**. The front lip portion **44** is angled downward towards the plane defined by the first main base edge **20** and the second main base edge **22**. In one aspect, the front lip portion **44** is angled (optionally, curved) downwards. In another aspect, the first upper foot **32** and the second upper foot **34** are curved or angled downward in the region of the front lip portion **44**. The angling/curving of the front lip portion **44** helps guide an overlying surgical drape, in this example an ophthalmic drape **46**, towards a position just above the patient's nose **48** and mouth **50** (See FIGS. 7 and 8) when the first lower foot **24** and the second lower foot **30** are positioned on the patient's chest **26**. In one particular aspect of the disclosure, the front lip portion **44** is angled downwards (optionally, curved downwards) at an angle of 30 degrees from the vertical axis **52**. In still another aspect, the front lip portion **44** only extends from the main crown **18** and not the first main base edge **20** or the second main base edge **22** (see FIG. 3). In other aspects, the front lip portion **44** may be angled based on the average size of a patient **28**.

(16) In another aspect of the disclosure, the surgical drape support **10** may include one or more accessory attachment features **54**. The accessory attachment features **54** allow medical monitoring and oxygen delivery lines to be secured to the surgical drape support **10**. In one aspect, at least one accessory attachment feature **54** is positioned on the inner main surface **14**. One accessory attachment feature **54** is contemplated to be an oxygen hose clamp securing an oxygen line **56** (see FIG. 8). The surgical drape support **10**, when placed on the patient **28** along with a surgical drape such as the ophthalmic drape **46**, creates a cavity **58** where oxygen may be delivered without direct contact with the patient **28**. This further minimizes stress on the patient **28**. In other aspects, the oxygen may be delivered directly to the patient through a cannula or other delivery device (see FIG. 9). The surgical drape support **10** defines at least four accessory pathways **60** to route medical accessories to the patient **28**. A main pathway **62** is defined through the main inner surface **14** of the main arch **12**. A secondary pathway **64** is defined between the first upper foot **32** and the second upper foot **34**. A first side pathway **66** is defined between the first lower foot **24** and the first upper foot **32**. A second side pathway **68** is defined between the second lower foot **30** and the second upper foot **34**. The use of multiple pathways allows accessories delivered to the patient **28** to be separated for organizational purposes. In addition, it allows accessories to be added or removed without disrupting accessories already installed.

(17) Referring now to FIGS. 6-9, depicting an aspect of the current disclosure, the surgical drape support **10** is placed on the patient's chest **26** (see FIG. 6). A surgical drape, such as ophthalmic drape **46**, is placed over the patient **28**. The surgical drape support **10** is positioned such that the ophthalmic drape **46** is angled from the surgical drape support **10** to just above the patient's nose **48**. The surgical drape support **10** can be moved up and down the patient's chest **26** until cavity **58** is sufficient for the comfort of the patient **28** while the angle of the ophthalmic drape **46** does not interfere with the surgeon's tools. If the patient's chest **26** is too small for the lower foot width **36**, the surgical drape support **10** can be inverted such that the first upper foot **32** and the second upper foot **34** are in contact with the patient's chest **26**. In at least one additional aspect of the disclosure, the first lower foot **24** and the second lower foot **30** as well as the first upper foot **32** and second upper foot **34** may be formed with a gripping pattern to prevent slippage of the ophthalmic drape

46. In other aspects the feet **24**, **30**, **32** and **34** may be coated with an anti-slippage material.

(18) Surgical drape supports according to the invention are expected to improve patient comfort and safety during any type of surgery to the head and neck, especially ophthalmic surgery as shown by way of example in this specification. Additional benefits include the ability to manufacture the surgical drape support by “additive” (3-D printing) techniques, which yields an affordable and versatile support which can be readily custom matched to any patient. The ability to invert the support to suit different patients and mount accessories (clips for oxygen lines, etc.) further increases the versatility of the design. The surgical drape support according to the invention may be formed of material, which is easy to sanitize, thereby furthering the economic advantages of the invention.

EXEMPLARY ASPECTS

(19) In view of the described products, systems, and methods and variations thereof, herein below are described certain more particularly described aspects of the invention. These particularly recited aspects should not however be interpreted to have any limiting effect on any different claims containing different or more general teachings described herein, or that the “particular” aspects are somehow limited in some way other than the inherent meanings of the language literally used therein.

(20) Aspect 1: A surgical drape support for positioning on a patient's chest to support a drape above the patient's nose and mouth, the surgical drape support comprising: a main arch defining an inner main surface, an outer main surface, a main crown, a first main base edge, and a second main base edge; a first lower foot extending downward from the first main base edge and away from the outer main surface; and a second lower foot extending downward from the second main base edge and away from the outer main surface; a first upper foot extending upwards and away from the outer main surface to a position above the main crown; and a second upper foot extending upwards and away from the outer main surface to a position above the main crown; wherein when the first lower foot and the second lower foot are positioned on the patient's chest, the first upper foot and the second upper foot are configured to support the drape over the patient's nose and mouth.

(21) Aspect 2: The surgical drape support of aspect 1, wherein when the first upper foot and the second upper foot are positioned on the patient's chest, the first lower foot and the second lower foot are configured to support the drape over the patient's nose and mouth.

(22) Aspect 3: The surgical drape support of any one of aspects 1 and 2, wherein the first lower foot and the second lower foot comprise concave surfaces when they are positioned on the patient's chest.

(23) Aspect 4: The surgical drape support of any one of aspects 1-3, wherein the first upper foot and the second upper foot comprise concave surfaces when they are positioned on the patient's chest.

(24) Aspect 5: The surgical drape support of any one of aspects 1-4, wherein the first lower foot and the second lower foot define a lower foot width; wherein the first upper foot and the second upper foot define an upper foot width; and wherein the upper foot width is smaller than the lower foot width.

(25) Aspect 6: The surgical drape support of aspect 5, wherein lower foot width is between 180 and 220 mm; and wherein the upper foot width is between 118 and 158 mm.

(26) Aspect 7: The surgical drape support of any one of aspects 1-5, wherein the main arch defines a main arch height; and wherein the main arch height is approximately 85 mm.

(27) Aspect 8: The surgical drape support of any one of aspects 1-7, further comprising: at least one accessory attachment feature.

(28) Aspect 9: The surgical drape support of aspect 8, wherein the at least one accessory attachment feature is positioned on the main inner surface.

(29) Aspect 10: The surgical drape support of any one of aspects 8 and 9, wherein the at least one accessory attachment feature comprises an oxygen hose clamp.

(30) Aspect 11: The surgical drape support of any one of aspects 1-10, further comprising at least four accessory pathways, the at least four accessory pathways comprising: a main pathway through the main inner surface; a secondary pathway between the first upper foot and the second upper foot; a first side pathway between the first upper foot and the first lower foot; and a second side pathway between the second upper foot and the second lower foot.

(31) Aspect 12: The surgical drape support of any one of aspects 1-11, wherein the main arch comprises a semi-cylinder.

(32) Aspect 13: The surgical drape support of any one of aspects 1-12, wherein the main arch is concave when the first lower foot and the second lower foot are positioned on the patient's chest; and wherein the main arch is convex when the first upper foot and the second lower foot are positioned on the patient's chest.

(33) Aspect 14: The surgical drape support of any one of aspects 1-13, wherein the main arch comprises: a symmetrical body portion defining a vertical axis; and a front lip portion, the front lip portion tapered downwards at an angle from the vertical axis the first lower foot and second lower foot are positioned on patient's chest.

(34) Aspect 15: The surgical drape support of aspect 14, wherein the front lip portion is angled downward from the vertical axis at an angle of approximately 30 degrees.

(35) Aspect 16: A surgical drape support for positioning on a patient's chest to support a drape above the patient's nose and mouth, the surgical drape support comprising: a main arch defining an inner main surface, an outer main surface, a main crown, a first main base edge, and as second main base edge; a first lower foot extending downward from the first main base edge and away from the outer main surface; and a second lower foot extending downward from the second main base edge and away from the outer main surface, the first lower foot and the second lower foot defining a lower foot width; a first upper foot extending upwards and away from the outer main surface to a position above the main crown; and a second upper foot extending upwards and away from the outer main surface to a position above the main crown, the first upper foot and the second upper foot defining an upper foot width, the upper foot width smaller than the lower foot width; wherein when the first lower foot and the second lower foot are positioned on the patient's chest, the first upper foot and the second upper foot are configured to support the drape over the patient's nose and mouth; wherein when the first upper foot and the second upper foot are positioned on the patient's chest, the first lower foot and the second lower foot are configured to support the drape over the patient's nose and mouth.

(36) Aspect 17: The surgical drape support of aspect 16, wherein the first lower foot and the second lower foot comprise concave surfaces when they are positioned on the patient's chest; and wherein the first upper foot and the second upper foot comprise concave surfaces when they are positioned on the patient's chest.

(37) Aspect 18: The surgical drape support of any one of aspects 16 and 17, wherein lower foot width is between 180 and 220 mm; and wherein the upper foot width is between 118 and 158 mm

(38) Aspect 19: The surgical drape support of any one of aspects 16-18, wherein the main arch defines a main arch height; and wherein the main arch height is approximately 85 mm.

(39) Aspect 20: The surgical drape support of any one of aspects 16-19, wherein upper foot width is approximately 70 percent of the lower foot width.

(40) Aspect 21: The surgical drape support of any one of aspects 16-20, further comprising: at least one accessory attachment feature.

(41) Aspect 22: The surgical drape support of aspect 21, wherein the at least one accessory attachment feature is positioned on the main inner surface.

(42) Aspect 23: The surgical drape support of any one of aspects 21-22, wherein the at least one accessory attachment feature comprises an oxygen hose clamp.

(43) Aspect 24: The surgical drape support of any one of aspects 16-23, further comprising at least four accessory pathways, the at least four accessory pathways comprising: a main pathway through

the main inner surface; a first side pathway between the first upper foot and the first lower foot; and a second side pathway between the second upper foot and the second lower foot.

(44) Aspect 25: The surgical drape support of any one of aspects 16-24, wherein the main arch comprises a semi-cylinder.

(45) Aspect 26: The surgical drape support of any one of aspects 16-25, wherein the main arch comprises: a symmetrical body portion defining a vertical axis; and a front lip portion, the front lip portion tapered downwards at an angle from the vertical axis the first lower foot and second lower foot are positioned on patient's chest.

(46) Aspect 27: The surgical drape support of aspect 26, wherein the front lip portion is angled downward from the vertical axis at an angle of approximately 30 degrees.

(47) Aspect 28: A method of supporting a surgical drape over a patient comprising: placing a surgical drape support on the patient's chest, the surgical drape support comprising: a main arch defining an inner main surface, an outer main surface, a main crown, a first main base edge, and a second main base edge; a first lower foot extending downward from the first main base edge and away from the outer main surface; and a second lower foot extending downward from the second main base edge and away from the outer main surface, the first lower foot and the second lower foot defining a lower foot width; a first upper foot extending upwards and away from the outer main surface to a position above the main crown; and a second upper foot extending upwards and away from the outer main surface to a position above the main crown, the first upper foot and the second upper foot defining an upper foot width, the upper foot width smaller than the lower foot width; positioning the first lower foot and the second lower foot on the patient's chest such that the surgical drape is positioned over the patient's nose and mouth.

(48) Aspect 29: The method according to aspect 28, further comprising: inverting the surgical drape support such that the first upper foot and the second upper foot are positioned on the patient's chest for smaller patients such that the drape over the patient's nose and mouth.

Claims

1. A surgical drape support configured for positioning on a patient's chest to support a drape above the patient's nose and mouth, the surgical drape support comprising: a main arch defining an inner main surface, an outer main surface, a main crown, a first main base edge, and a second main base edge; a first lower foot extending downward from the first main base edge and away from the outer main surface; and a second lower foot extending downward from the second main base edge and away from the outer main surface; a first upper foot extending upwards and away from the outer main surface to a position above the main crown; and a second upper foot extending upwards and away from the outer main surface to a position above the main crown; wherein when the first lower foot and the second lower foot are positioned on the patient's chest, the first upper foot and the second upper foot are configured to support the drape over the patient's nose and mouth.

2. The surgical drape support of claim 1, wherein the first lower foot and the second lower foot define a lower foot width; wherein the first upper foot and the second upper foot define an upper foot width; and wherein the upper foot width is smaller than the lower foot width.

3. The surgical drape support of claim 2, wherein lower foot width is between 180 and 220 mm; and wherein the upper foot width is between 118 and 158 mm.

4. The surgical drape support of claim 3, wherein the main arch defines a main arch height; and wherein the main arch height is approximately 85 mm.

5. The surgical drape support of claim 1, further comprising: at least one accessory attachment feature.

6. The surgical drape support of claim 5, wherein the at least one accessory attachment feature comprises an oxygen hose clamp.

7. The surgical drape support of claim 1, wherein the main arch comprises: a symmetrical body

portion defining a vertical axis; and a front lip portion, the front lip portion tapered down wards at an angle from the vertical axis when the first lower foot and second lower foot are positioned on patient's chest.

8. The surgical drape support of claim 7, wherein the front lip portion is angled downward from the vertical axis at an angle of 30 degrees.

9. The surgical drape support of claim 1, wherein when the first upper foot and the second upper foot are positioned on the patient's chest, the first lower foot and the second lower foot are configured to support the drape over the patient's nose and mouth.

10. A surgical drape support configured for positioning on a patient's chest to support a drape above the patient's nose and mouth, the surgical drape support comprising: a main arch defining an inner main surface, an outer main surface, a main crown, a first main base edge, and a second main base edge; a first lower foot extending downward from the first main base edge and away from the outer main surface; and a second lower foot extending downward from the second main base edge and away from the outer main surface, the first lower foot and the second lower foot defining a lower foot width; a first upper foot extending upwards and away from the outer main surface to a position above the main crown; and a second upper foot extending upwards and away from the outer main surface to a position above the main crown, the first upper foot and the second upper foot defining an upper foot width, the upper foot width smaller than the lower foot width; wherein when the first lower foot and the second lower foot are positioned on the patient's chest, the first upper foot and the second upper foot are configured to support the drape over the patient's nose and mouth; wherein when the first upper foot and the second upper foot are positioned on the patient's chest, the first lower foot and the second lower foot are configured to support the drape over the patient's nose and mouth.

11. The surgical drape support of claim 10, further comprising: at least one accessory attachment feature.

12. The surgical drape support of claim 11, wherein the at least one accessory attachment feature comprises an oxygen hose clamp.

13. The surgical drape support of claim 10, wherein the main arch comprises: a symmetrical body portion defining a vertical axis; and a front lip portion, the front lip portion tapered downwards at an angle from the vertical axis the first lower foot and second lower foot are positioned on patient's chest.

14. The surgical drape support of claim 13, wherein the front lip portion is angled downward from the vertical axis at an angle of 30 degrees.

15. The surgical drape support of claim 10, wherein the first lower foot and the second lower foot comprise concave surfaces; and wherein the first upper foot and the second upper foot comprise concave surfaces.

16. The surgical drape support of claim 10, wherein lower foot width is between 180 and 220 mm; and wherein the upper foot width is between 118 and 158 mm.

17. The surgical drape support of claim 10, wherein the upper foot width is 70 percent of the lower foot width.

18. The surgical drape support of claim 10, wherein the main arch comprises a semi-cylinder.

19. A method of supporting a surgical drape over a patient comprising: placing a surgical drape support on the patient's chest, the surgical drape support comprising: a main arch defining an inner main surface, an outer main surface, a main crown, a first main base edge, and a second main base edge; a first lower foot extending downward from the first main base edge and away from the outer main surface; a second lower foot extending downward from the second main base edge and away from the outer main surface, the first lower foot and the second lower foot defining a lower foot width; a first upper foot extending upwards and away from the outer main surface to a position above the main crown; and a second upper foot extending upwards and away from the outer main surface to a position above the main crown, the first upper foot and the second upper foot defining

an upper foot width, the upper foot width smaller than the lower foot width; and positioning the first lower foot and the second lower foot on the patient's chest such that the surgical drape is positioned over the patient's nose and mouth.

20. The method according to claim 19, further comprising: inverting the surgical drape support such that the first upper foot and the second upper foot are positioned on the patient's chest for smaller patients such that the drape over the patient's nose and mouth.
